



February 23, 2026

Steven Posnack

Principal Deputy Assistant Secretary for Technology Policy

Assistant Secretary for Technology Policy / Office of the National Coordinator for Health
Information Technology

U.S. Department of Health and Human Services

RE: HHS Health Sector AI RFI, RIN 0955-AA13

Dear Deputy Assistant Secretary Posnack,

Community Care Cooperative (C3) appreciates the opportunity to respond to the Department of Health and Human Services' Request for Information on accelerating the adoption and use of artificial intelligence (AI) in the health sector.

C3 is a non-profit, Federally Qualified Health Center (FQHC)-owned Accountable Care and Management Services Organization. Founded, governed, and operated by community health centers. We support more than 50 FQHCs across fifteen states and the District of Columbia and we operate Medicare, Medicaid, and commercial value-based payment arrangements covering more than 250,000 lives. As a primary-care-led organization, our mission is to leverage the collective strengths of FQHCs to improve health outcomes for underserved communities while lowering total cost of care.

In our role as a Management Services Organization, C3 works closely with health centers to understand their operational, clinical, and technological needs, including serving as a leader and group purchaser for both clinical and administrative AI solutions. Through this work, we have direct experience supporting the real-world deployment of AI tools in under-resourced primary care settings. We have seen how thoughtfully implemented AI can reduce administrative burden, enhance patient access, and support high-quality chronic disease management, particularly in areas such as documentation, patient communication, and revenue cycle functions, provided that adoption strategies are practical, equitable, and aligned with strong safeguards and accountability.

Our comments focus on two areas:

1. How HHS can accelerate applied AI innovation in primary care and community health, and
2. The regulatory and governance safeguards necessary to maintain patient trust and support responsible adoption.



Accelerating Applied AI Innovation in Primary Care and Community Health

Primary care is the only specialty consistently shown to reduce total cost of care through prevention, early detection, and longitudinal chronic disease management. Yet it remains chronically under-resourced and under-invested, particularly in community health centers, which serve approximately 14 percent of the U.S. population while receiving a small share of federal health care funding. As a result, AI adoption strategies designed for well-resourced health systems often do not translate effectively to safety-net settings.

To realize the potential of AI to improve outcomes and advance health equity, primary care providers - and particularly FQHCs - must be treated as essential partners in the design, validation, and deployment of AI solutions, not simply as downstream adopters.

Where Applied AI Is Delivering Immediate Value

Across our network, health centers have expressed strong interest in AI tools that deliver near-term operational and clinical value, including:

- **Reducing administrative burden**, through ambient documentation, coding and billing support, and workflow automation that improves accuracy and sustainability while alleviating staff burnout.
- **Enhancing patient access and experience**, including AI-enabled voice agents that support appointment navigation, language access, and patient communication.
- **Supporting high-quality clinical care**, particularly tools that augment clinical workflows, such as pre-visit preparation and documentation, while preserving clinician judgment and accountability.

Importantly, trust is foundational to adoption. Health centers and their patients must be confident that AI tools are designed to improve access to care and understanding of health information, not to obscure decision-making or introduce new risks.

Barriers to Adoption in Safety-Net Settings

Despite growing interest, FQHCs face meaningful barriers to AI adoption, including:

- **Upfront and ongoing costs**, coupled with uncertainty around return on investment in value-based environments.



- **Workflow disruption and training demands**, particularly in settings with limited staffing capacity.
- **Interoperability challenges**, including integration across EHR instances and versions and avoidance of siloed, proprietary tools.
- **Limited internal capacity** to vet, govern, and continuously monitor AI solutions.

These barriers are structural, not philosophical, and they reflect the day-to-day realities of operating in safety-net primary care. With the right federal policy levers, they are addressable.

Federal Levers to Enable Practical Adoption

In response to Questions 2 and 8, we encourage HHS to consider the following actions:

- **Align payment models with AI adoption costs**, including embedding technology adoption incentives into CMMI models such as the LEAD and ACCESS models, and allowing advanced or onboarding investments to offset implementation and management costs.
- **Leverage trusted intermediaries and dissemination hubs** that can convene technology developers and community health centers, reduce duplication, and accelerate real-world validation and learning (for example, collaboratives that pair early-stage developers with safety-net providers, such as ATTUNE-like models, or implementation-focused hubs such as Health AI Partnerships).
- **Create mutual incentives for data sharing and interoperability**, supporting bidirectional data flow between providers and developers while avoiding data silos that undermine care coordination and population health management.

Data Representativeness

FQHCs care for patients who are, on average, sicker, lower-income, and more socially complex than many other populations. AI tools developed without meaningful inclusion of safety-net data risk working well for some populations while failing others- and, in the worst cases, exacerbating existing disparities. Representative data, ongoing monitoring, and corrective action are essential to ensuring AI advances equity rather than undermining it. Community health centers should be viewed as indispensable partners in developing trustworthy, equitable AI.

Regulation, Governance, and Necessary Safeguards



Maintaining patient trust is paramount to the successful adoption of AI in clinical and administrative settings. AI has real potential to improve care delivery, but only if it is deployed with strong guardrails.

First, **human accountability must be preserved**. AI tools should augment - not replace - clinical judgment, and qualified human providers must remain responsible for final clinical decisions.

Second, **data privacy and transparency are essential**. We urge HHS to:

- Ensure strict adherence to HIPAA and Privacy Act requirements for all AI tools handling protected health information.
- Require clear, explicit disclosure of data use, sharing, and permissions by AI developers.
- Develop a vendor evaluation or certification-like framework analogous to EHR certification, building on emerging work such as the AI Vendor Disclosure Framework developed by Health AI Partnerships, that enables providers to assess privacy protections, data governance practices, and trustworthiness with confidence.

Such guardrails would reduce uncertainty, support responsible adoption, and protect both patients and providers.

Closing

C3 strongly supports HHS' efforts, through ONC and CMMI, to expand access, reduce administrative burden, and sustain high-quality primary care through the responsible use of AI. As a primary-care-led, FQHC-owned organization with hands-on deployment experience, C3 and the community health centers we support stand ready to partner with HHS on future pilots, learning collaboratives, and stakeholder engagement. We would welcome the opportunity to inform implementation efforts with on-the-ground experience from primary care and community health settings.

Thank you for the opportunity to share our perspective.

Respectfully submitted,

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Community Care Cooperative